

NEIGRIHMS SHILLONG  
Department of Neurology & Cognitive Sciences

Mawdiangdiang, Shillong, Meghalaya 793018

CONFIDENTIAL CLINICAL MEDICAL RECORD - NEUROLOGY CONSULTATION

|                                                                       |                                                                           |                                                       |
|-----------------------------------------------------------------------|---------------------------------------------------------------------------|-------------------------------------------------------|
| <b>Patient Name:</b> Nongrum, Mary                                    | <b>MRN:</b> NEIG-2026-4412                                                | <b>Sex:</b> Female                                    |
| <b>DOB / Age:</b> 10/11/1957 (Age: 68)                                | <b>Date of Exam:</b> 23/08/2026                                           | <b>Primary Caregiver:</b> Daphisha Nongrum (Daughter) |
| <b>Attending Neurologist:</b> Dr. Wanroi Khonglah, MD, DM (Neurology) | <b>Referring Physician:</b> Dr. Patricia Lyngdoh, MD (Geriatric Medicine) | <b>Clinical Status:</b> Outpatient Evaluation         |

REASON FOR EVALUATION

Evaluation for progressive short-term memory difficulties and word-finding pauses noted over the past 14 months.

CLINICAL HISTORY (HISTORY OF PRESENT ILLNESS)

The patient is a 68-year-old right-handed Khasi female, former school teacher and active community elder in Shillong, presenting with a 14-month history of subtle memory lapses. She reports frequently misplacing grocery lists, forgetting recently read Bible verses, and needing reminder notes for social appointments. Functional Assessment: Fully independent in basic ADLs and complex IADLs (cooking traditional meals, managing household budget, navigating Shillong city taxis, taking medications with a weekly pill organizer). No difficulties with driving or domestic chores. Behavioral Symptoms: Mild anxiety regarding cognitive decline; no apathy, agitation, social withdrawal, or sleep disturbances reported.

| PAST MEDICAL HISTORY                                                                                                                                                                                                | ACTIVE MEDICATIONS                                                                                                                                                                           |
|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <ul style="list-style-type: none"><li>Primary Hypothyroidism (well-managed on Levothyroxine 50 mcg)</li><li>Osteoarthritis bilateral knees</li><li>No prior stroke, traumatic brain injury, or depression</li></ul> | <ul style="list-style-type: none"><li>Levothyroxine 50 mcg PO daily (fasting)</li><li>Calcium Carbonate 500 mg + Vitamin D3 daily</li><li>Paracetamol 650 mg PO PRN for joint pain</li></ul> |

PHYSICAL & NEUROLOGICAL EXAMINATION

Alert, exceptionally well-groomed, highly articulate, fully oriented. Cranial nerves intact. Motor: 5/5 strength throughout, normal tone. Reflexes: 2+ symmetric. Gait: Normal, no bradykinesia or instability. Romberg negative.

STANDARDIZED COGNITIVE TESTING

Mini-Mental State Examination (MMSE): Score 27/30 | Montreal Cognitive Assessment (MoCA): Score 24/30

- Orientation (Time/Place): 10/10 (Fully oriented)
- Registration: 3/3
- Attention & Calculation: 5/5
- Recall: 1/3 (Delayed recall impaired on MoCA: 2/5 words, improves with category cueing)
- Language & Visual Construction: 8/9 (Clock Drawing intact; minor word-retrieval hesitations on phonemic fluency: 9 words in 1 min)

DIAGNOSTIC WORKUP & NEUROIMAGING BIOMARKERS

- Laboratory: Normal CBC, CMP, Free T4, TSH (2.1 mIU/L), Vitamin B12 (610 pg/mL), ESR normal.
- Brain MRI (Without Contrast): Mild symmetrical hippocampal volume reduction within expected bounds for age (MTA Score: Grade 1). No significant cortical atrophy. No white matter hyperintensities (Fazekas Grade 0). No ischemic lesions.

DIAGNOSTIC IMPRESSION

- Mild Cognitive Impairment, Amnesic Single-Domain (aMCI) (ICD-10: G31.84).
- Objective memory deficit on standardized delayed recall with fully preserved functional autonomy in daily life and absence of overt dementia.

TREATMENT PLAN & CLINICAL RECOMMENDATIONS

- Non-Pharmacological: Structured cognitive stimulation, daily brisk walking around Ward's Lake, Mediterranean-DASH brain-healthy diet.
- Lifestyle & Monitoring: Advised maintaining active social roles and diary-keeping. Cholinesterase inhibitors not indicated at this stage.
- Cardiovascular Risk Control: Maintain optimal thyroid and metabolic parameters.
- Follow-Up: Formal repeat neuropsychological testing and clinical review in 6 months.

Electronically Verified & Signed by:

Dr. Wanroi Khonglah, MD, DM (Neurology)  
Associate Professor of Neurology | Reg No: MCI-58291  
NEIGRIHMS, Shillong, Meghalaya

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